



---

**Beyond Good Intentions: How Schools and Workplaces Use  
Evidence to Navigate Health and Wellbeing Investment  
Decisions—A Scoping Review**

|                  |                                                                    |
|------------------|--------------------------------------------------------------------|
| Journal:         | <i>International Journal of Organization Theory &amp; Behavior</i> |
| Manuscript ID    | IJOTB-05-2025-0150                                                 |
| Manuscript Type: | Research Paper                                                     |
| Keywords:        | Well-Being, Implementation, Workplace Health Promotion Programs    |
|                  |                                                                    |

SCHOLARONE™  
Manuscripts

1  
2  
3  
4  
5  
6  
7  
8  
9  
10  
11  
12  
13  
14  
15  
16  
17  
18  
19  
20  
21  
22  
23  
24  
25  
26  
27  
28  
29  
30  
31  
32  
33  
34  
35  
36  
37  
38  
39  
40  
41  
42  
43  
44  
45  
46  
47  
48  
49  
50  
51  
52  
53  
54  
55  
56  
57  
58  
59  
60

# Beyond Good Intentions: How Schools and Workplaces Use Evidence to Navigate Health and Wellbeing Investment Decisions—A Scoping Review

**Abstract**

**Purpose**

Although there have been increased pushes to promote health and wellbeing promotion in workplaces and schools, little is known about the way these organisations make use of evidence to support their health and wellbeing investment decisions. This is important, as evidence plays a role in influencing organisational decision-making processes about wellbeing.

**Design/methodology**

A scoping review was conducted to explore how workplaces and schools use evidence in their wellbeing investments decisions, and to synthesise findings across a range of disciplines and OECD countries. A total of 27 articles from nine OECD countries were included and analysed thematically.

**Findings**

Both studies of schools and workplaces faced challenges for identifying and using a variety of forms of evidence, due to insufficient time and human resources. Needs for expertise and relevance to organisational interests also led schools and workplaces to prioritise the use of certain forms of evidence over others. Finally, findings emphasized the role of power interests in shaping how wellbeing related evidence was selected and used (or not) by organisational decision-makers.

**Originality**

The review highlights how evidence use affects the selection of wellbeing initiatives and provide information on how and why organisations choose to invest in wellbeing initiatives. It provides nuances to a sole problem-solving model approach, showing that resources and organisational power dynamics shape evidence use in decision-making processes about wellbeing.

**Key words:** health and wellbeing; evidence use; schools; workplaces; decision-making; resource allocation; power.

## 1. Background

A significant amount of an individual's lifetime can be spent in schools and at work, making these places essential for health and wellbeing promotion. However, little is known about how these organisations make use of evidence to support their health and wellbeing investment decisions. As research has shown, cultures of evidence use vary through sectors and contexts (Lorenc et al., 2014), and further research is needed on how and why particular types of evidence and knowledge become influential in specific contexts (Turner et al., 2022).

There have been increased global pushes to promote health and wellbeing promotion in workplaces and schools. For instance, new guidelines were published by the World Health Organization (WHO) to recommend actions to tackle risks to mental health, such as stressful work environments and heavy workloads (WHO, 2022). Similarly, UNESCO now actively encourages schools to implement a 'whole-school approach' to promote the mental well-being of children and young people (WHO and UNESCO, 2019). However, while education research and research-based evidence constitute a clear area of focus for school decision-makers (Coburn and Penuel, 2016), evidence related to health and wellbeing may fall outside their domain of usual expertise, which results in increasing pressure for spending limited resources of money in these specific areas.

Regarding workplaces, there had been a growing number of wellbeing programmes being implemented by employers in recent years, after the Covid-19 pandemic, to support the mental and physical health of their employees (OECD, 2022). However, while wellbeing initiatives are often showcased as a source of pride for progressive human resource departments (Barry et al., 2024), there is limited information available about how such programmes are being implemented. This is especially true regarding the role of evidence use and its influence in organisational decision-making processes, which may affect the overall effectiveness of wellbeing initiatives (Daniels et al., 2021). According to Mueller et al. (2007: 859), organisational decision-making process are shaped by socio-political elements, which they define as 'decision-makers doing excessive analysis for the purpose of persuasion and communication.' The selection (or not) of different forms of evidence by decision-makers can be part of this work of analysis, making it a particular area of interest to understand the health and wellbeing investments in workplaces. This is also important, since research on evidence use in organisations are key parts of organisational learning processes, yet, have in majority focussed on improving organisational effectiveness and performance.

Evidence can be defined as ‘information that enables actors to judge whether a proposition is true or not’ (Buse et al, 2024, p.177). In organisational contexts, evidence-based management (or EBM) has been defined as the ‘systematic collection and judicious use of the best available evidence from multiple sources when making organizational decisions’ (Criado-Perez et al, 2023: 1653). However, what is considered ‘evidence’ relates to what counts as ‘knowledge’, which varies, depending on epistemological, political and moral perspectives (Learmonth, and Harding, 2006). For Turner et al (2022), rather than adopting a problem-solving approach, evidence use is often shaped by the interactions between individuals and their organisational context and influenced by organisational traditions as well as politics and power relations.

In this review we make a distinction between evidence which is produced by external research bodies, and decisions based on internal experiences in the organisations, because we are interested in how research-generated evidence is utilised in schools and workplaces. Concerns have been raised over the risk to define use of research evidence through a sole managerialist lens (Learmonth and Harding, 2006). Given the important role played by schools and workplaces for health and wellbeing promotions, and the ambiguous role that such evidence plays in affecting decision-making processes, this scoping review aims to bring a better understanding of how evidence is currently used (or not) in schools and workplaces to select health and wellbeing initiatives.

**2. Methods**

A scoping review was used to address the research question ‘How do workplaces and schools use evidence to allocate resources for health and wellbeing initiatives?’ A three-stage procedure was used for the scoping review. This comprised ‘*planning*’, ‘*execution*’, and ‘*reporting*’, a procedure critically assessed and used by Tranfield et al., (2003) to be applicable in the field of social sciences and organisations studies, where we anticipated studies would be located. This reporting of the scoping review followed the extended PRISMA guidelines for scoping review (Annexe 1) (Tricco et al., 2018).

In ‘*planning*’ the review, we selected *Assia*, *Sage Health Journals*. *Scopus* and *PubMed* as search engines to ensure a multi-disciplinary focus on organisations, evidence use, and health. To be included, studies needed to focus on health and/or wellbeing promotion, report empirical findings, include some discussion of allocation of the organisation’s financial, human, or space resources, and some mention of the use (or non-use) of information or evidence in reaching

investment (resource allocation) decisions (Table 1). Studies were excluded if: they did not take place in a workplace or school context; they were focused only on the effects of health and/or mental health intervention (and not the decision-making process behind these interventions); they were not in English, and finally, they did not take place in an OECD country. This last aspect was because the research team aimed to focus on contexts similar to the UK context, in which the research team and project are based.

[Insert Table 1 here]

In ‘*executing*’ the search, we combined categories of words related to (i) wellbeing (e.g. health, mental health) ; (ii) the organisation (e.g., schools, students, workplaces, employees); (iii) investments (e.g., promotion, initiatives), and (iv) evidence (e.g., information). An example of a search strategy on *Scopus* can be found in Annexe 2. We also adopted a forward search literature technique which focuses on retrieving relevant original articles or works cited by a study or relevant document (Couteau, 2014). This enables the researcher to identify relevant studies based on database searches and those obtained through expert knowledge of those publishing in the field.

For the initial research stage, the lead author reviewed all titles and abstracts for relevance, according to the inclusion and exclusion criteria. Three additional raters reviewed a random selection of 5% of titles and abstracts for both workplaces and schools, to assess accuracy of reviews by the lead author. After screening of abstracts and titles we used a categorisation process to assess the remaining manuscripts for full-text review. Studies were initially sorted into different groups, based on the proximity to the research question (Table 2) (Roberts et al., 2002). Studies which were labelled as A1 were included as they met all inclusion criteria for the review.

[Insert Table 2 here.]

A data extraction template was used to extract information on (a) characteristics of the study (authors, date of publication, country); (b) study setting (type of school/workplace), health and wellbeing focus; (c) study research methods (e.g., interviews, case studies, secondary data analysis etc.); (d) type of resources allocated (e.g., staff, funding, time etc.); (e) individuals

1  
2  
3  
4  
5  
6  
7  
8  
9  
10  
11  
12  
13  
14  
15  
16  
17  
18  
19  
20  
21  
22  
23  
24  
25  
26  
27  
28  
29  
30  
31  
32  
33  
34  
35  
36  
37  
38  
39  
40  
41  
42  
43  
44  
45  
46  
47  
48  
49  
50  
51  
52  
53  
54  
55  
56  
57  
58  
59  
60

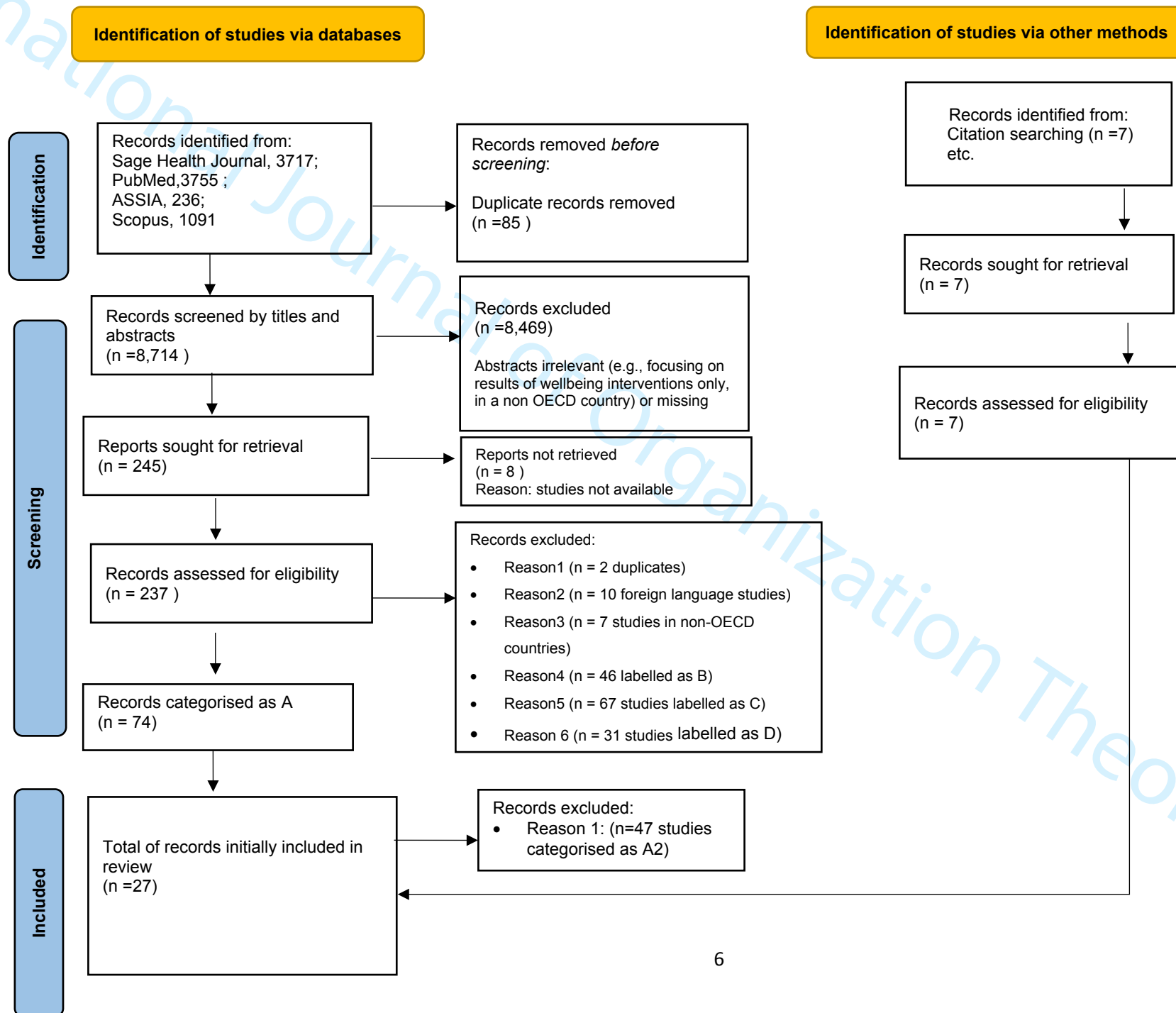
making decisions (e.g., teachers, students, employers, etc.); (f) type of evidence used/discussed (e.g., external research data, internal data, etc.); and (g) the ways in which evidence was used.

Following data extraction, a thematic analysis was undertaken to analyse the findings. Themes were developed by the lead author who has a background in sociology and organisation studies, in collaboration with co-authors. The themes were developed both inductively and deductively to explain the types of evidence used, and the ways in which evidence was used, with the support of the literature on the use of evidence in decision-making processes.

**3. Results**

8,714 studies were retrieved after removal of duplicates, for both schools and workplaces (see Figure 1). After screening the abstracts and titles, 245 studies were considered of adequate relevance for categorisation and full text assessment. A very small number of discrepancies between reviewers were identified and resolved through discussions between the lead reviewer and other co-authors. Those discrepancies related to the way evidence was used (or not) in the studies to inform decision-making processes. After full text review, 74 of the 245 studies were categorised as A (due to the proximity to the research question), of which 27 were categorised as A1 (14 school-based studies and 13 workplace-based studies) and were included in the review.

[insert Figure about 1 here – PRISMSA Diagram for the research stage]



1  
2  
3  
4  
5  
6  
7  
8  
9  
10  
11  
12  
13  
14  
15  
16  
17  
18  
19  
20  
21  
22  
23  
24  
25  
26  
27  
28  
29  
30  
31  
32  
33  
34  
35  
36  
37  
38  
39  
40  
41  
42  
43  
44  
45  
46



### 3.2 Details of studies

Details of the 14 school-based studies are summarised in Table 3. Studies took place between 2010 and 2024 and in Australia, Austria, Canada, the UK, France, Ireland, and the USA. Details of the 13 workplace-based studies are summarised in Table 4. Studies took place between 2005 and 2021 in Australia, Canada, Germany, Sweden, the UK, and the USA.

For schools, all but one study focused on the implementation of programmes and activities related to students' health and wellbeing. The remaining study reported on the introduction of sexual health teaching. All schools invested in staff resources, training or use of existing staff time to support students' wellbeing. 36% reported explicitly the use of money to invest in wellbeing programmes with 14% specifying investment in technological equipment and 14% highlighting time investment in development of curriculum content. In addition, some schools facilitated access to a skilled health promotion stakeholder (Higgins and Booker, 2022, Philips and Martinez, 2010).

The key decision-makers related to resources were school leaders, principals, or head teachers. The role of the decision-maker was influenced by the context and traditions in which the study was taking place. For instance, one study in France (Pommier et al., 2011) emphasised that due to 'the history of the French school system', the role of parents as decision-makers was not well-recognised. Conversely, a study taking place in the context of England emphasised the influence of school governors on the school decision-making process, some of them being parents (Morris and Dobson., 2021).

For workplaces, all studies focused on the implementation or promotion of health, safety and mental health initiatives. In one study, health and safety was explicitly prioritised over mental health (Pavlista et al., 2021). Larsson et al (2016) suggested that countries adopted different definitions of workplace health promotion, with health promotion tending to be considered as individuals 'wellness' and 'health behaviour' in the US while in European countries, a broader view of workplace health promotion incorporated the quality of the work environment and conditions as important factors for employee wellbeing. In some US studies, however, the resources allocated to health and wellbeing promotion relied as much on employees' input and needs regarding their work conditions and experiences, through trade unions and wellness committees than 'health packages' proposed by insurance brokers (Hugues et al., 2011; Witt et al., 2013).

For studies on workplaces, there was some variation over the role and importance of decision-makers. A majority focused on the role of senior management as key decision-makers. However, the importance of other key actors varied by country. For example, in Sweden, there was a specific emphasis on employee participation (Larsson et al., 2016), and this was presented as a legal requirement. This emphasis on employee participation was also present in another study located in Germany. In studies based in the UK, US and Canada, decision-making seemed to rely primarily on senior managers (Witt et al., 2013; Hughes et al., 2011; Nelson et al., 2015), and the use of employees' feedback was dependent on the presence and power of the voice of employees' representatives (Hughes et al., 2011; Nelson et al., 2015; Witt et al., 2013).

[Insert Tables 3 and 4 here]

3.2 Types of evidence used in schools and workplaces

Three broad types of evidence used in the studies were identified. First, '*external research data*' which included forms of evidence produced outside the school or organisation, such as academic research on effectiveness or cost-effectiveness of programmes. Second, '*internally generated data*', which is evidence generated within the school or workplace organisation. This included informal evaluation of initiatives as well as research findings produced through survey, interviews, and observations within the organisation; Third, '*external expertise*' which constitutes information provided by stakeholders outside the school or organisation with expert skills or knowledge influencing the school or organisation's decision-making (e.g., public health partners, consultants, etc). Study participants within the organisations (teachers, managers, workers) also reported the use of their skills, experiences and insights to make decisions.

To inform their decisions, all school studies reported using their skills, knowledge or experiences to make decisions. 36% of studies used these in combination with external research data, 43% with the support of internally collected data (e.g., survey, informal evaluation), and 57% with inputs from external experts. Many school studies (70%) highlighted restricted human and time resources to support access to varied forms of evidence. For example, one study (Adamowitsch et al., 2017) mentioned that teachers lacked time and financial resources to take part in the evidence-gathering, and therefore it was down to the head teacher to find

time to look for research or scientific evidence related to health and wellbeing, use internal data, and make decisions based on what was found. Other studies emphasised a lack of specialist staff who could investigate evidence about health, for example school nurses (Cygan et al., 2020). Higgins and Booker (2022) noted that schools had to rely on their own funds to hire external experts, such as mental health speakers.

In workplace studies, most organisations reported using their insights and experiences to make a decision (77%). However, the use of external experts was more commonly described in studies of workplaces than in studies of schools. For instance, 77% of workplace studies used external expertise such as consultants, compared with 57% of schools, which could be linked to a question of financial resources. Like schools' studies, however, resources for identifying and using evidence were often limited. Many studies mentioned insufficient human resources, lack of internal skills to analyse data systematically, or inappropriate survey instruments to collect internal data upon which to base their decisions (Pavlista et al., 2021; Quirk et al., 2018; Larsson et al., 2016; Miller, and Haslam, 2009).

We generated three themes based on the data extracted to explain how schools and workplaces use evidence to invest in health and wellbeing. These are

- (i) *Evidence-based provision adapted to context*, which looks at how evidence is used or not used in tandem with context of the decision;
- (ii) *Having expertise*, which reports decision-makers seeking 'expertise' in their selection of evidence or reliance on their experiences to make a decision related wellbeing;
- (iii) *Challenging the status quo*, which shows instances where evidence could be used to challenge decision-making or current wellbeing practice.

[Include Tables 5 and 6]

#### *Evidence-based provision adapted by context*

A key emerging theme was the importance of organisational context of schools and workplaces. Context appeared to be an important moderator for the use of different forms of evidence. For example, studies highlighted the importance of teachers' insight in adapting expert-recommended health and wellbeing activities to their diverse classrooms, so these

activities would be relevant to their students' experiences (Ozer et al., 2011; Higgins and Booker, 2022). A second school-based study highlighted the fact that teachers needed critically review the evidence available to them and decide which forms of evidence were the most likely to be 'actionable' in their classroom context (Pommier et al, 2011).

In workplaces, there was a similar emphasis on the importance of context. Actors from industry sector associations in the Marsh et al (2018) study argued that there was a need to re-shape the language and appropriateness of health promotional materials, business cases and resources according to the industry sector they represented. In Leso et al (2024), the success of health-related interventions relied on leaders' abilities to engage with their workforce to identify and specifically target priority areas, such as ergonomic risks and injuries for care workers, noise risks in the construction sector, etc. Pavlista, et al (2021) Rohlman et al. (2018), and Witt et al., (2013) emphasised that decision-makers often looked for wellbeing activities or programmes relevant for the size of their organisation but could often struggle in finding appropriate evidence and tools. For example, in Pavlista et al (2021) the lack of an appropriate health and safety survey for small organisations was a problem for collecting data.

The context could sometimes constrain the use of a variety of evidence. In one school-based study, religious beliefs influenced the way schools allocated resources to their sexual health teaching courses (Phillips and Martinez, 2010). Other school studies (Cygan et al., 2020; Adamowitsch et al., 2017) also emphasised the importance of cultural practices and traditions in a particular context which may hinder or facilitate the implementation of new health practices. It was noted that the broader institutional context also played a role in directing the use of certain forms of evidence over others, such as cost-benefit evidence. In both workplaces and schools, studies in the US, UK and Ireland highlighted the costs and benefits of interventions as the most important forms of evidence for decision-makers (Miller and Haslam, 2009; Hughes et al., 2011; Leso et al., 2024), which could be linked to governmental requirements in regards to certain forms of evidence (e.g., such as costs-benefits evidence).

### *Having expertise*

The second theme describes tensions over what is defined as 'expertise' by decision-makers in their selection of evidence or reliance on their experiences to make a decision. In many cases, schools' reliance on their professional skills, experience, and insights were essential to implement health initiatives (Meiklejohn et al, 2021; Adamowitsch et al, 2017; Brown, et al,

2018), such as putting in place their own internal referral systems (Higgins, and Booker, 2022). Yet, findings also crucially emphasised the need to access external resources, such as guidance and support from external professionals, and the efforts from schools to bring in speakers on mental health, even though access was often limited due to a lack of money (Higgins and Booker, 2022).

Challenges around the constitution of ‘expertise’ also emerged in workplaces. A perceived lack of ‘expertise’ (e.g., human resources) meant that internal data were often not analysed extensively (Larsson et al., 2016; Rohlman et al., 2018). Miller and Haslam (2009) highlighted a perceived lack of robust empirical data to accompany business cases about investment in health in organisations, with a preference for ‘intuitive, emotional and ethical arguments’ to drive organisational actions. When health related information was framed as ‘specialist knowledge’, however, decision-makers often felt they lacked time or legitimacy to analyse organisational data related to their employees’ health status. This discourse about a lack of organisational ‘expertise’ for using evidence meant that workplaces often used shortcuts, such as ‘easy’ and ‘low-cost’ interventions, as noted by Rohlman et al (2018), or calling external experts to deliver talks or transfer information (Quirk et al., 2018; Marsh et al., 2018; Witt et al., 2013; Nelson et al., 2015). Training possibilities for workers was also another opportunity to increase organisational expertise, such as workers’ knowledge and expertise about the health and safety risks of their occupation, but the Leso et al (2024) review indicates that such training was often not sustained continuously. On the other hand, using external ‘expertise’, such as external consultants, could also be a way to depoliticise sensitive issues. For example, calling in experts was not only seen as an ‘easy’ way to promote health and wellbeing, but also as a ‘neutral way’ to increase workers’ acceptance of interventions (Pavlista et al., 2021).

### *Challenging the status quo*

Finally, evidence could also be used to challenge decision-making, or employers’ or headteachers’ decision-making. For example, Adamowitsch et al (2017) note that the headteacher could become the target of persuasion strategies by other staff to encourage them to look at different forms of evidence (Cygan et al., 2020) or ideas (Adamowitsch et al., 2017) in relation to resource allocation and the implementation of health interventions (e.g.,

convincing the headteacher to allocate money for a specific intervention). Evidence for wellbeing interventions also helped challenge the academic-based performance focus, according to some studies (Higgins and Booker, 2022).

In workplaces, Baril-Gingras and Dubois-Ouellet (2018) and Hall et al (2006) demonstrated that knowledge about health and safety (such as specialist research on health and work conditions from books, web sources and personal and professional contacts) could be transformed into 'knowledge activism' by trade union and employee representatives to push for improvements in health and safety practices in their workplaces. This 'alternative use of evidence' by employees could then encourage management to allocate resources to new safety areas (e.g., investing money in more preventive equipment). In Hall et al (2006), a health and safety committee gathered specialised clinical knowledge about the danger of a lack of ventilation and lighting when using machines, pushing the company to invest large sums of money in a new ventilation system.

## 4. Discussion

### 4.1 Key Findings

Findings from this review demonstrate that schools and workplaces' actors seek evidence to develop their health and wellbeing expertise while addressing organisational needs and interests. Findings also highlight that evidence utilisation and search for expertise is however constrained by power dynamics and hierarchies of decision-making processes.

### 4.2 Comparaison to existing literature and explanation of findings:

- (i) *Pulling evidence in schools and workplaces' context: a need for expertise which fit organisational needs*

Institutional, organisational, and political processes shape schools' and workplaces' practices, such as the prioritisation of some forms of evidence over others to inform their decisions about health and wellbeing investments. In this aspect, the findings of this review echo other research focusing on priority setting in specific organisational contexts demonstrating how institutional, political, and cultural factors play a role in influencing decision-making processes (Lessard et al., 2009;2020; Eddama and Coast, 2008). Findings from this review demonstrated that schools



and workplaces prioritised looking for evidence which fitted their needs for ‘expertise’ – or lack of – and was relevant to their organisational interests.

Schools relied heavily on their insights and experiences to inform and justify their decision-making. Experience operated as a moderator of other forms of research and external evidence. Schools’ needs for evidence which was relevant for their needs echo other studies on third sector organisations such as Hardwick et al (2015) and the preference for these organisations to rely on locally collected information, including their shared values and experiential knowledge. Other schools, however, highlighted the importance of external expertise, but struggled to access state-funded external professional agencies, and had restrained resources to fund access to their own external experts (Higgins and Booker, 2022).

For workplaces, although many employers engaged with their internal data, studies showed that they often felt they lacked the necessary ‘expertise’ (e.g., human resources) to make efficient decisions about health and wellbeing investments in the context of their organisation.) An adequate understanding of one’s context is often necessary for gaining expertise (Johri, 2015), especially about a particular area such as health and wellbeing. The lack of expertise felt by workplace decision-makers echoes Kowalski and Loretto (2017) who point out the challenges of adopting a holistic approach to wellbeing in the workplace due to limited resources and priority given to focus on employees’ performance, rather than their wellbeing needs.

(ii) *‘Technical evidence’ as political knowledge*

We also found that evidence could be used as way to challenge or advocate for a particular intervention in some instances, transforming a technical form of knowledge into a form of ‘political knowledge’ to push for changes (Carlan et al., 2022), rather than seeing evidence as a sole managerialist tool (Learmonth and Harding, 2006). As noted by Turner et al (2022:1076), further research is needed on how ‘professional power and conflict inform, and shape evidence use in decision-making.’ Such power interests and conflicts were clearly present in how schools and workplaces used evidence. The hierarchical nature of the school-setting meant that headteachers or principals were the main decision-makers. One way to subvert this power asymmetry in the decision-making process in schools was to ‘convince’ or ‘persuade’ headteachers, framing evidence as appealing to their needs and preferences. One study emphasized the ‘emotional labour’ that health administrators had to perform in their organisations when introducing evidence to decision-makers (Blackburn, and Apesoa-Varano,

2019). Similarly, evidence gathered about the importance of students' health and wellbeing could help challenge the over-emphasis on academic-based performance (Higgins and Booker, 2022).

Likewise, in workplace studies, there were divergent power interests between employers and employees. While the use of external evidence (such as involving external consultants) could be framed to 'neutralise' this power divergence, the alternative use of evidence and consideration of employees' voices could also help challenge management position and divert the allocation of resources to the benefit of employees (Hall et al., 2006; Baril Gingras, and Dubois-Ouellet (2018)). Consequently, rather than being a 'threat' to democratic processes and pluralism in workplaces (Learmonth and Harding, 2006), the use of evidence from different parts of the organisation may on the contrary potentially reinforce the possibility for reinforced democratic forms of decision-making in the workplace and contribute to positive politics (Parmentier and Reynaud, 2021). In this way, our findings contribute to a better understanding of how the development of technical knowledge can enhance existing knowledge and support activism (Carlan et al., 2022), for instance by representing an opportunity to bargain for collaborative evidence-based health and safety initiatives in the workplace and better consistency of approaches across workplaces and sectors.

#### 4.3 Implications of findings

Our findings enable a crucial understanding of the role played by 'evidence' in shaping decision-making processes in workplaces and schools regarding health and wellbeing initiatives. This is especially important, since the Covid-19 has provoked a renewed interest in wellbeing from the perspective of workplaces and schools. Our review highlights the important political role of evidence use in affecting organisational decision-making and demonstrates how recognising multiple forms of evidence in organisations could potentially improve transparency and democracy of decision-making processes in this area. On a practical aspect, researchers producing evaluations about wellbeing initiatives should consider the best way their evaluations can be made available to various organisational actors taking part in the decision-making process (managers but also employees' representatives, for instance) and how the context and dynamics of these organisations enables them to access and use different forms of evidence. For example, this might take the form of policy-driven initiatives to build capacity for understanding and interpreting the implications of wellbeing programmes for use in schools



and workplaces, and to consider the effects of combining different forms of evidence for implementing effective wellbeing innovations in schools and workplaces.

#### 4.4 Limitations and Strengths

Our review has some limitations. The number of studies does not allow us to generalise the way schools and workplaces use evidence. Many of our workplace studies focused on small and medium businesses, in which the process of decision-making as well as availability of human and time resources may be different from larger organisations. Similarly, there are clearly a wide range of types of schools that are funded and managed in different ways across as well as within countries. The selected studies did not explore differences between diverse types of schools. The empirical studies in this review did not directly focus on resource allocation or evidence use as their primary objective, and this was reflected in the fragmented extraction.

On the other hand, we were able to synthesise these findings thematically, identifying key areas needed for further work in wellbeing investment evidence in schools and workplaces. Finally, our findings only reflect processes of evidence-use in workplaces and schools for wellbeing activities in the context of OECD countries.

#### 4.5 Further work

The review findings offer some further avenues to explore. For example, there is a need to explore in more depth how specific political and institutional contexts influence workplaces and schools' use of evidence in their decision-making processes, in and beyond OECD countries. This has implications for what types of interventions or innovations they invest in and can inform how appropriate those investments are for the given setting, such as large workplaces or specific industrial sectors.

Similarly, given that schools or workplaces may prioritise their internal knowledge to inform their wellbeing and health investment decisions, more research is needed to understand how schools and workplaces value and prioritise different types of evidence, and what are the barriers and opportunities existing around the use of external evidence.

### 4. Conclusion

Schools', and workplaces' use of evidence about health and wellbeing are shaped by their contextual needs, what they perceive as 'expertise', and the power interests at play between decision-makers and the rest of the organisation. There are implications for the role of the

research community, as research produced to improve health and wellbeing in these settings needs to be sensitive to how these important factors influence decision-making processes.

## References

Adamowitsch, M., Gugglberger, L., and Dür, D. (2017) 'Implementation practices in school health promotion: findings from an Austrian multiple-case study', *Health Promotion International*, 32(2): 218–230.

Agnew, N.M., Ford, K.M. and Hayes, P.J. (1997), "Expertise in context: personally constructed, socially selected and reality-relevant?", in Feltovich, P.J., Ford, K.M. and Hoffman, R.R. (Eds), *Expertise in Context: Human and Machine*, MIT/AAAI Press, Cambridge, MA: 219-244.

Baril-Gingras, G. and Dubois-Ouellet, S. (2018) 'Framing, Resources and Repertoire of Local Trade Union Action for Health and Safety: A Study Conducted with a Quebec Central Labour Body' *Relations Industrielles*, 73(3): 429.

Blackburn, S. and Apesoa-Varano, E. C. (2019) 'Brokering Student Well-Being: Understanding the Work of School Health Administrators', *The Journal of School Nursing*, 35(6): 412–421

Breheny K, Frew E, Williams I, Passmore S, Coast J (2020). 'Use of Economic Evidence When Prioritising Public Health Interventions in Schools: A Qualitative Study with School Staff.' *International Journal of Environmental Research and Public Health*, 17(23): 9077.

Brown, K. M. *et al.* (2018) "'Now What?'" Perceived Factors Influencing Knowledge Exchange in School Health Research', *Health Promotion Practice*, 19(4): 590–600.

Buse, K., Mays, N., Colombini, M., Fraser, A., Khan, M., & Walls, H. (2024). *Making Health Policy*, 3e. McGraw Hill.

Carlan N, Szymanski T, Van Zetten J, Hilbrecht M, Bigelow P. (2022). The Path from Survey Development to Knowledge Activism: A Case Study of the Use of a Physical Loads Survey in

a Retail Workplace. *NEW SOLUTIONS: A Journal of Environmental and Occupational Health Policy*, 32(1):65-76.

Cluff, L. A., Lang, J.E., Rineer, J.R., Jones-Jack, N.H., and Strazza, K.M. (2018). 'Training Employers to Implement Health Promotion Programs: Results From the CDC Work@Health® Program.' *American Journal of Health Promotion*, 32(4):1062-1069.

Coburn, C. E., & Penuel, W. R. (2016). Research-Practice Partnerships in Education: Outcomes, Dynamics, and Open Questions. *Educational Researcher*, 45(1), 48–54.  
<http://www.jstor.org/stable/43996895>

Cole, D. C., et al. (2005). 'Quality of working life indicators in Canadian health care organizations: A tool for healthy, health care workplaces?' *Occupational Medicine* 55(1): 54-59.

Couteau, O. (2014) Forward searching – A complement to keyword- and class-based patentability searches, *World Patent Information*, 37: 33-38, ISSN 0172-2190.

Cygan, H., Tribbia, C. and Tully, J. (2020) 'School Health Policy Implementation: Facilitators and Challenges.' *The Journal of School Nursing*, 36(5):330–338.

Daniels, K., Watson, D., Nayani, R., Tregaskis, O., Hogg, M., Etuknwa, A., Semkina, A. (2021) Implementing practices focused on workplace health and psychological wellbeing: A systematic review, *Social Science & Medicine*, Volume 277, 113888.

Eddama, O., Coast J. (2009) Use of economic evaluation in local health care decision-making in England: A qualitative investigation. *Health Policy*, 89(3): 261-70.

Hall, A., Forrest, A., Sears, A. and Carlan, N. (2006). 'Making a Difference: Knowledge Activism and Worker Representation in Joint OHS Committees.' *Relations Industrielles*, 61(3): 408-436,532-533.

Hardwick, R., Anderson, R. and Cooper, C. (2015) 'How do third sector organisations use research and other knowledge? A systematic scoping review' *Implementation Sciences*, 10: 84.

Higgins, E. & Booker, R. (2022) 'The implementation of a whole school approach to mental health and well-being promotion in the Irish primary school context.' *Health education journal*. 81 (6), 649–666.

Hughes, M. C. *et al.* (2011) 'Understanding the Decision-Making Process for Health Promotion Programming at Small to Midsized Businesses', *Health Promotion Practice*. 12(4): 512–521.

Jessiman, P.E., Campbell, R., Jago, R. *et al.* (2019). 'A qualitative study of health promotion in academy schools in England.' *BMC Public Health*, 19:1186.

Johri, A. (2015). Impressions in action: the socially situated construction of expertise in the workplace. *Journal of Organizational Ethnography*, 4(1): 44-63.

Kowalski, T., Loretto, W. (2017) 'Well-being and HRM in the changing workplace.' *The International Journal of Human Resource Management*, 28(16): 2229-2255.

Larsson, R., *et al.* (2016). 'Managing workplace health promotion in municipal organizations: The perspective of senior managers.' *Work*, 53(3) : 485-498

Le, L. K. D., Esturas, A. C., Mihalopoulos, C., Chiotelis, O., Bucholc, J., Chatterton, M. L., & Engel, L. (2021). 'Cost-effectiveness evidence of mental health prevention and promotion interventions: A systematic review of economic evaluations', *PLoS medicine*, 18(5): e1003606.

Learmonth, M. and Harding, N (2006). 'Evidence-based management: the very idea', *Public Administration*, 84(2): 245-266.

Leso, V. *et al.* (2024) The Total Worker Health® (TWH) approach: a systematic review of its application in different occupational settings. *BMC public health*. [Online] 24 (1), 2037–25.

Lessard, C., *et al.* (2010). 'The role (or not) of economic evaluation at the micro level: Can Bourdieu's theory provide a way forward for clinical decision-making?' *Social Science & Medicine*, 70(12): 1948-1956.

1  
2  
3 Lorenc T, Tyner EF, Petticrew M, Duffy S, Martineau FP, Phillips G, et al. Cultures of evidence  
4 across policy sectors: systematic review of qualitative evidence. *Eur J Pub Health*.  
5 2014;24(6):1041–7.  
6  
7

8  
9  
10 Marsh, G., et al. (2018). ‘Workplace wellness: industry associations are well placed and some  
11 are ready to take a more active role in workplace health’, *BMC Health Services Research*,  
12 18(1): 565.  
13  
14

15  
16  
17 Meiklejohn, S., Peeters, A. and Palermo, C. (2021) ‘Championing Health Promoting Schools:  
18 A secondary school case study from Victoria, Australia’, *Health Education Journal*, 80(2):  
19 187–198.  
20  
21

22  
23 Miller, P. and C. Haslam (2009). ‘Why employers spend money on employee health:  
24 Interviews with occupational health and safety professionals from British Industry’, *Safety*  
25 *Science*, 47(2): 163-169.  
26  
27

28  
29  
30 Morris, R. and Dobson, G. (2021) ‘Spending the pupil premium: What influences leaders’  
31 decision-making?’, *Educational Management Administration & Leadership*, 49(2): 284–302.  
32  
33

34  
35 Nelson, C. C., et al. (2015). ‘Integrating health promotion and occupational safety and health  
36 in manufacturing worksites: Perspectives of leaders in small-to-medium sized businesses’,  
37 *Work*, 52(1): 169-176.  
38  
39

40  
41  
42 Nutley, S. M., Walter, I & Davies, H. (2007). *Using evidence: How research can inform public*  
43 *services*, Bristol University Press.  
44  
45

46  
47 Ozer, E.J., Wanis, M.G. & Bazell, N. (2010) ‘Diffusion of School-Based Prevention Programs  
48 in Two Urban Districts: Adaptations, Rationales, and Suggestions for Change’, *Prevention*  
49 *Sciences*, 11:42–55.  
50  
51

52  
53 Patalay, P., Giese, L., Stanković, M., Curtin, C., Moltrecht, B. and Gondek, D. (2016), ‘Mental  
54 health provision in schools: priority, facilitators and barriers in 10 European countries’, *Child*  
55 *and Adolescent Mental Health*, 21: 139-147.  
56  
57  
58  
59  
60

Pavlista, V., et al. (2021). 'Barriers and drivers of psychosocial risk assessments in German micro and small-sized enterprises: a qualitative study with owners and managers' *BMC Public Health*, 21(1): 1376.

Parmentier, T. & Reynaud, Pr. E. (2022) "Can politics really be beneficial?" Toward a model for positive politics through consensus. *International journal of organization theory and behavior*. [Online] 25 (1/2), 56–71.

Pescud, M., Teal, R., Shilton, T. *et al.* (2015). 'Employers' views on the promotion of workplace health and wellbeing: a qualitative study' *BMC Public Health*, 15: 642.

Phillips, K. P., & Martinez, A. (2010). 'Sexual and Reproductive Health Education: Contrasting Teachers', Health Partners' and Former Students' Perspectives', *Canadian Journal of Public Health / Revue Canadienne de Sante Publique*, 101(5): 374–379.

Pommier, J., Guevel, M. and Jourdan D., 2011. 'A health promotion initiative in French primary schools based on teacher training and support: actionable evidence in context' *Global health promotion*, 18(1):34-38.

Quirk, H., et al. (2018). 'Barriers and facilitators to implementing workplace health and wellbeing services in the NHS from the perspective of senior leaders and wellbeing practitioners: a qualitative study', *BMC Public Health*, 18(1):1362

Roberts, T., Henderson, J., Mugford, M., Bricker, L., Neilson, J. and Garcia, J. (2002), 'Antenatal ultrasound screening for fetal abnormalities: a systematic review of studies of cost and cost effectiveness.' *BJOG: An International Journal of Obstetrics & Gynaecology*, 109: 44-56.

Rohlman, D. S., et al. (2018). 'What Could Total Worker Health® Look Like in Small Enterprises?' *Annals of Work Exposures and Health*, 62: s34-s41.

Schmidt, M., Werbrouck, A., Verhaeghe, N., Putman, K., Simoens, S., & Annemans, L. (2020). 'Universal mental health interventions for children and adolescents: a systematic review of health economic evaluations', *Applied health economics and health policy*, 18: 155-175.



Stevenson, D. and Farmer, P (2017) *Thriving at work: the Stevenson and Farmer review of mental health and employers*. Available at: [Thriving at Work: the Stevenson/Farmer review on mental health and employers \(publishing.service.gov.uk\)](https://thrivingatwork.service.gov.uk/)

Storey, K.E., Montemurro, G., Flynn, J., Schwartz, M., Wright, E., Osler, J., Veugelers, P., and Roberts, E. (2016) 'Essential conditions for the implementation of comprehensive school health to achieve changes in school culture and improvements in health behaviours of students', *BMC Public Health*, 16: 11-33.

Sulz, L. et al. (2016) 'Complexity of choice: Teachers' and students' experiences implementing a choice-based Comprehensive School Health model', *Health Education Journal*, 75(8): 986-997

Tranfield, D., Denyer, D., Smart, P. (2003). 'Towards a Methodology for Developing Evidence-Informed Management Knowledge by Means of Systematic Review', *British Journal of Management*, 14(3), 207-222.

Tricco, AC, Lillie, E, Zarin, W, O'Brien, KK, Colquhoun, H, Levac, D, Moher, D, Peters, MD, Horsley, T, Weeks, L, Hempel, S et al. (2018) 'PRISMA extension for scoping reviews (PRISMA-ScR): checklist and explanation.' *Annals of Internal Medicine*. 169(7):467-473.

Turner, S., D'Lima, D., Sheringham, J., Swart, N., Hudson, E., Morris, S., and Fulop, J. (2022) 'Evidence use as sociomaterial practice? A qualitative study of decision-making on introducing service innovations in health care', *Public Management Review*, 24:7, 1075-1099.

Turner, S., D'Lima, D., Hudson, E., Morris, S., Sherringam, J., Swart, N., and Fulop, N. (2017). 'Evidence use in decision-making on introducing innovations: a systematic scoping review with stakeholder feedback', *Implementation Science*, 12(1): 145.

Williams, I., and Stirling B. (2007). 'Understanding the limited impact of economic evaluation in health care resource allocation: A conceptual framework.' *Health Policy*, 80 (1):135-143.

Witt, L. B., et al. (2013). 'Motivating factors for small and midsize businesses to implement worksite health promotion.' *Health Promotion Practice*, 14(6): 876-884.



1  
2  
3  
4  
5  
6  
7  
8  
9  
10  
11  
12  
13  
14  
15  
16  
17  
18  
19  
20  
21  
22  
23  
24  
25  
26  
27  
28  
29  
30  
31  
32  
33  
34  
35  
36  
37  
38  
39  
40  
41  
42  
43  
44  
45  
46  
47  
48  
49  
50  
51  
52  
53  
54  
55  
56  
57  
58  
59  
60

World Health Organization (2015). *The European Mental Health Action Plan, 2013-2020*:  
[https://www.euro.who.int/\\_data/assets/pdf\\_file/0020/280604/WHO-Europe-Mental-Health-Acion-Plan-2013-2020.pdf](https://www.euro.who.int/_data/assets/pdf_file/0020/280604/WHO-Europe-Mental-Health-Acion-Plan-2013-2020.pdf)

World Health Organization (2022) *WHO and ILO call for new measures to tackle mental health issues at work, joint news release*. Available at: <https://www.who.int/news/item/28-09-2022-who-and-ilo-call-for-new-measures-to-tackle-mental-health-issues-at-work>

World Health Organization and UNESCO (2019). *Global Standards for Health Promoting Schools: Available at:*  
[https://www.who.int/maternal\\_child\\_adolescent/adolescence/globalstandards-for-health-promoting-schools-who-unesco.pdf](https://www.who.int/maternal_child_adolescent/adolescence/globalstandards-for-health-promoting-schools-who-unesco.pdf)

**Figures and Tables**

*Table 1 Key concepts and corresponding inclusion and exclusion criteria*

| Concept                                                               | Included                                                                                                                                                                                                                                                                                                                                    | Excluded                                                                           |
|-----------------------------------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------|
| <b><i>Schools and Workplaces</i></b>                                  | For schools, any studies taking place in an academy, institution, college, training, university, or education setting. For workplaces, any study taking place in organisation(s); business(es); corporations; companies, public, private or third sector, charities, worksites, workspace, workplace, place of work or place of employment. | Healthcare organisation describing health investments for patients.                |
| <b><i>Resources-allocation for health and wellbeing promotion</i></b> | Studies which mention some discussion of priority-setting and, decision-making, about allocation of resources (staffing, money, space) for health and mental wellbeing promotion in a school or workplace context.                                                                                                                          | Studies focusing only on effects or results of health and wellbeing interventions. |
| <b><i>Evidence</i></b>                                                | Studies which mention using evidence to inform (or not to inform) decision-making.                                                                                                                                                                                                                                                          | Studies not mentioning the use of evidence to inform decision-making.              |

Table 2. Categorisation criteria of selected studies

| Initial categorisation of studies                                                                                                                                                                                                                                                        | Further categorisation of studies                                                                                                                                                                                                                                                                                                                                                                                                                                             |
|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| A. Studies focus on <i>schools/workplaces</i> , discuss <i>health and mental wellbeing promotion and the role of evidence</i> .                                                                                                                                                          | <ol style="list-style-type: none"> <li>1. In-depth discussion of the role of evidence in decision-making about <i>activities</i> and how <i>resources</i> for these activities are allocated and informed by health and mental wellbeing promotion activities.</li> <li>2. Mention evidence only superficially and do not elaborate on the way report on researchers' views (theoretical, conceptual, guidelines) on the way it is used to inform decision-making.</li> </ol> |
| B. Studies focus on <i>schools/workplaces</i> and <i>health and mental wellbeing promotion and evidence</i> . However, these studies do not rely on empirical evidence.                                                                                                                  |                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
| C. Studies focus on <i>schools/workplaces</i> and <i>health and mental wellbeing promotions</i> , examine how resources are allocated but do not discuss evidence OR vice-versa, (studies report on evidence use but do not discuss decision-making about resources allocation).         |                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
| D. Studies focus on <i>health and wellbeing promotion activities</i> , but do not focus on <i>schools/workplaces</i> , do not consider <i>allocation of resources</i> to these activities and/or do not consider <i>evidence</i> to support allocation of resources to these activities. |                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |

1  
2  
3  
4  
5  
6  
7  
8  
9  
10  
11  
12  
13  
14  
15  
16  
17  
18  
19  
20  
21  
22  
23  
24  
25  
26  
27  
28  
29  
30  
31  
32  
33  
34  
35  
36  
37  
38  
39  
40  
41  
42  
43  
44  
45  
46  
47  
48  
49  
50  
51  
52  
53  
54  
55  
56  
57  
58  
59  
60

Table 3- Details of school studies

| Authors (date of publication) | Setting                                    | Health and wellbeing focus                                                                                                                  | Research methods   | Type of resources allocated                  | Decision-makers                                                                                                           |
|-------------------------------|--------------------------------------------|---------------------------------------------------------------------------------------------------------------------------------------------|--------------------|----------------------------------------------|---------------------------------------------------------------------------------------------------------------------------|
| Adamowitsch et al (2017)      | Austria- primary and secondary schools     | Health promotion<br>Health behaviour                                                                                                        | Interviews         | Staff                                        | Headteachers, health coordinators, teachers, municipality representatives                                                 |
| Blackburn et al (2019)        | Canada- public schools                     | Coordination of practice across external health services for schools                                                                        | Interviews         | Staff, funding                               | School board, superintendents, leaders, principals, executive                                                             |
| Breheny et al (2020)          | England- unspecified school                | Physical activity                                                                                                                           | Interviews         | Staff, curriculum content                    | Teachers, leadership staff                                                                                                |
| Brown et al (2018)            | Canada- unspecified school                 | Health behaviour                                                                                                                            | Interviews         | Staff                                        | Teachers, vice-principals                                                                                                 |
| Cygan et al (2020)            | U.S.A-Primary schools                      | Physical activity and nutrition policies                                                                                                    | Interviews         | Staff and funding                            | Teachers, principal, School Health Administrators                                                                         |
| Jessiman et al (2019)         | England- Academies                         | Health behaviour                                                                                                                            | Interviews         | Staff, staff training, technology equipment, | Academy and school leaders                                                                                                |
| Meiklejohn et al(2021)        | Australia-Secondary school                 | Health behaviour                                                                                                                            | Case study         | Staff                                        | Teachers, students, principals,                                                                                           |
| Morris et al(2021)            | England-Unspecified school                 | No defined focus                                                                                                                            | Interviews         | Staff, funding                               | School leaders, school governors                                                                                          |
| Ozer et al(2010)              | U.S.A-Secondary schools                    | Wellbeing                                                                                                                                   | Interviews         | Staff, technology equipment                  | Teachers, program developers                                                                                              |
| Phillips et al (2010)         | Canada- Public and Catholic schools        | Sexual health                                                                                                                               | Interviews         | Staff, curriculum content                    | Teachers                                                                                                                  |
| Pommier et al (2011)          | France-Primary schools                     | Health behaviour                                                                                                                            | Case study         | Staff                                        | School representatives, parents' representatives, regional actors                                                         |
| Storey et al (2016)           | Canada-Unspecified school                  | School health                                                                                                                               | Secondary analysis | Staff, funding                               | School health facilitators, teachers, school principals                                                                   |
| Higgins and Booker (2022)     | Ireland-3 primary and post-primary schools | Mental Health and Wellbeing promotion<br>Access to external professional services, and access to evidence-based interventions and practices | Case studies       | Staff, funding                               | principal, vice-principal, Home School Community Liaison Co-Ordinator, assistant psychologist and family support workers. |

|                   |                           |                   |                        |       |          |
|-------------------|---------------------------|-------------------|------------------------|-------|----------|
| Sulz et al (2016) | Canada-Unspecified school | Health behaviours | Focus group interviews | Staff | Teachers |
|-------------------|---------------------------|-------------------|------------------------|-------|----------|

| Authors (date of publication) | Setting                             | Health and wellbeing focus                          | Research methods | Type of resources                                         | Decision-makers                                                 |
|-------------------------------|-------------------------------------|-----------------------------------------------------|------------------|-----------------------------------------------------------|-----------------------------------------------------------------|
| Baril-Gingras et al (2018)    | Canada- multiple sectors            | Health and safety promotion                         | Interviews       | Time for emplo representatives                            | Trade union leaders/representatives, employers                  |
| Coyle et al (2005)            | Canada- health                      | Quality of work life                                | Case studies     | Budget allocations                                        | Human resources (HR) teams                                      |
| Hall et al (2006)             | Canada- auto parts plants           | Health and safety promotion                         | Interviews       | Money, staff                                              | Managers, trade union representatives                           |
| Hughes, et al (2011)          | U.S.A- multiple sectors             | Health insurers informing health promotion programs | Interviews       | Time and money                                            | Employers, HR managers, trade union                             |
| Larsson, et al (2016)         | Sweden- municipal government        | Health and safety promotion                         | Interviews       | Time                                                      | HR, senior managers                                             |
| Marsh, et al (2018)           | Australia- multiple sectors         | Health and wellbeing                                | Interviews       | Information                                               | Industry association representatives                            |
| Miller et al (2009)           | U.K- multiple sectors               | Occupational health and wellbeing promotion         | Interviews       | Money                                                     | Occupational health and safety professionals                    |
| Nelson, et al (2015)          | U.S.A- multiple sectors             | Workplace health promotion                          | Interviews       | Money, Staff                                              | HR, occupational health specialists, senior managers, employees |
| Pavlista et al (2021)         | Germany- small and medium companies | Psychosocial risks prevention                       | Interviews       | Limited financial and human resources                     | Employers and managers                                          |
| Quirk, et al (2018)           | U.K-Public hospital                 | Health and wellbeing promotion                      | Interviews       | Limited financial and human resources, government funding | Senior leaders, wellbeing practitioners, HR                     |

|                             |                                                                        |                                                        |                         |                                                                                              |                                                                          |
|-----------------------------|------------------------------------------------------------------------|--------------------------------------------------------|-------------------------|----------------------------------------------------------------------------------------------|--------------------------------------------------------------------------|
| <b>Rohlman et al (2018)</b> | U.S.A- small and medium companies                                      | Health and wellness promotion                          | Case studies            | Limited financial and human resources                                                        | Safety managers, and HR directors                                        |
| <b>Witt et al (2013)</b>    | U.S.A- small and medium companies                                      | Health promotion                                       | interviews              | Money, staff,                                                                                | Executives, internal health and wellness committees, senior HR personnel |
| <b>Leso et al (2024)</b>    | U.S.A- Mining, construction real estate, healthcare and other sectors. | Organisational based Total Workers' Health initiatives | Secondary data analysis | Time for meetings, having sufficient staff to participate in the programs, funding, workload | Leaders, HR managers and workers                                         |

*Table 4- Details of workplace studies*

1  
2  
3  
4  
5  
6  
7  
8  
9  
10  
11  
12  
13  
14  
15  
16  
17  
18  
19  
20  
21  
22  
23  
24  
25  
26  
27  
28  
29  
30  
31  
32  
33  
34  
35  
36  
37  
38  
39  
40  
41  
42  
43  
44  
45  
46

| Authors<br>(date of publication)    | Context                            | Types of evidence and experiences used |                           |                    |                                  | Way in which evidence and experiences were used |                   |                             |
|-------------------------------------|------------------------------------|----------------------------------------|---------------------------|--------------------|----------------------------------|-------------------------------------------------|-------------------|-----------------------------|
|                                     |                                    | External research data                 | Internally generated data | External expertise | Experiences, and insights skills | Evidence-based provision adapted by context     | To have expertise | To challenge the status quo |
| Meiklejohn et al (2021)             | Australia-public school            | Y                                      | N                         | N                  | Y                                | Y                                               | N                 | N                           |
| Adamowitsch et al(2017)             | Austria-Primary, secondary schools | N                                      | Y                         | N                  | Y                                | Y                                               | Y                 | Y                           |
| Brown, et al (2018)                 | Canada-schools unspecified         | N                                      | Y                         | Y                  | Y                                | Y                                               | N                 | N                           |
| Blackburn et al (2019)              | Canada-public schools              | N                                      | N                         | Y                  | Y                                | Y                                               | Y                 | N                           |
| Philips et al(2010)                 | Canada-public and Catholic schools | N                                      | N                         | Y                  | Y                                | Y                                               | N                 | N                           |
| Storey et al (2016)                 | Canada-school unspecified          | N                                      | Y                         | N                  | Y                                | Y                                               | Y                 | N                           |
| Sulz, et al (2016)                  | Canada-school unspecified          | Y                                      | N                         | N                  | Y                                | Y                                               | N                 | N                           |
| Breheny et al (2020)                | England-mix of schools             | N                                      | N                         | N                  | Y                                | Y                                               | Y                 | N                           |
| Jessiman, et al (2019)              | England-academies                  | N                                      | N                         | Y                  | Y                                | Y                                               | Y                 | N                           |
| Morris et al 2021)                  | England-mix of schools             | N                                      | Y                         | N                  | Y                                | Y                                               | Y                 | N                           |
| Pommier, Guével, and Jourdan (2011) | France-primary schools             | Y                                      | Y                         | Y                  | Y                                | Y                                               | Y                 | N                           |



|                                  |                                        |   |   |   |   |   |   |   |
|----------------------------------|----------------------------------------|---|---|---|---|---|---|---|
| <b>Cygan et al (2020)</b>        | U.S.A primary schools' context         | N | N | Y | Y | Y | Y | Y |
| <b>Ozer, E.J., et al (2010)</b>  | U.S.A-Secondary schools                | Y | N | Y | Y | Y | N | N |
| <b>Higgins and Booker (2022)</b> | Irish Primary and Post primary schools | Y | Y | Y | Y | Y | Y | Y |

*Table 5. Evidence and experiences use in school-based studies*

**Y=Yes; N=No**

Table 6. Evidence and experiences use in workplaces studies.

| Authors (date of publication) | Context                         | Evidence and experiences |                           |                    |                                  | Use of evidence and experiences             |                   |                             |
|-------------------------------|---------------------------------|--------------------------|---------------------------|--------------------|----------------------------------|---------------------------------------------|-------------------|-----------------------------|
|                               |                                 | External research data   | Internally generated data | External expertise | Experiences, insights and skills | Evidence-based provision adapted to context | To have expertise | To challenge the status quo |
| Marsh, et al (2018)           | Australia-Industry Associations | Y                        | N                         | N                  | N                                | Y                                           | N                 | N                           |
| Baril-Gingras et al (2018)    | Canada- multiple sectors        | Y                        | Y                         | Y                  | Y                                | Y                                           | Y                 | Y                           |
| Cole et al (2005)             | Canada- Health                  | Y                        | Y                         | N                  | Y                                | Y                                           | Y                 | N                           |
| Hall, Forrest et al (2006)    | Canada- auto plants             | Y                        | Y                         | Y                  | Y                                | Y                                           | Y                 | Y                           |
| Pavlista et al (2021)         | Germany-Small employers         | N                        | Y                         | Y                  | Y                                | Y                                           | N                 | N                           |
| Larsson, et al (2016)         | Sweden-Municipal government     | N                        | Y                         | Y                  | Y                                | Y                                           | N                 | N                           |
| Miller, et al (2009)          | U.K- multiple sectors           | N                        | Y                         | N                  | Y                                | Y                                           | N                 | N                           |
| Quirk et al (2018)            | UK-National Health Service      | N                        | Y                         | Y                  | Y                                | Y                                           | Y                 | N                           |
| Hughes, et al (2011)          | U.S.A- multiple sectors         | N                        | N                         | Y                  | N                                | Y                                           | N                 | N                           |

|                              |                                                                 |   |   |   |   |   |   |   |
|------------------------------|-----------------------------------------------------------------|---|---|---|---|---|---|---|
| <b>Nelson et al (2015)</b>   | U.S.A-Manufacturing industries                                  | N | N | Y | Y | Y | N | N |
| <b>Rohlman, et al (2018)</b> | U.S.A-Small and medium companies                                | N | Y | Y | N | Y | N | N |
| <b>Witt et al (2013)</b>     | U.S.A- small/medium employers                                   | N | Y | Y | Y | Y | N | N |
| <b>Leso et al (2024)</b>     | Mining, construction real estate, healthcare and other sectors. | Y | Y | Y | Y | Y | Y | Y |

Y=Yes; N=No

## ) Annexes

### Annexe 1

#### Preferred Reporting Items for Systematic reviews and Meta-Analyses extension for Scoping Reviews (PRISMA-ScR) Checklist

| SECTION            | ITEM | PRISMA-ScR CHECKLIST ITEM                                                                                                                                                                                                     | REPORTED ON PAGE # |
|--------------------|------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|--------------------|
| <b>TITLE</b>       |      |                                                                                                                                                                                                                               |                    |
| Title              | 1    | Identify the report as a scoping review.                                                                                                                                                                                      | 1                  |
| <b>ABSTRACT</b>    |      |                                                                                                                                                                                                                               |                    |
| Structured summary | 2    | Provide a structured summary that includes (as applicable): background, objectives, eligibility criteria, sources of evidence, charting methods, results, and conclusions that relate to the review questions and objectives. | 1                  |

| INTRODUCTION                      |   |                                                                                                                                                                                                                                                                           |     |
|-----------------------------------|---|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----|
| Rationale                         | 3 | Describe the rationale for the review in the context of what is already known. Explain why the review questions/objectives lend themselves to a scoping review approach.                                                                                                  | 1-3 |
| Objectives                        | 4 | Provide an explicit statement of the questions and objectives being addressed with reference to their key elements (e.g., population or participants, concepts, and context) or other relevant key elements used to conceptualize the review questions and/or objectives. | 3   |
| METHODS                           |   |                                                                                                                                                                                                                                                                           |     |
| Protocol and registration         | 5 | Indicate whether a review protocol exists; state if and where it can be accessed (e.g., a Web address); and if available, provide registration information, including the registration number.                                                                            | 3-4 |
| Eligibility criteria              | 6 | Specify characteristics of the sources of evidence used as eligibility criteria (e.g., years considered, language, and publication status), and provide a rationale.                                                                                                      | 3-4 |
| Information sources*              | 7 | Describe all information sources in the search (e.g., databases with dates of coverage and contact with authors to identify additional sources), as well as the date the most recent search was executed.                                                                 | 3-4 |
| Search                            | 8 | Present the full electronic search strategy for at least 1 database, including any limits used, such that it could be repeated.                                                                                                                                           | 3-4 |
| Selection of sources of evidence† | 9 | State the process for selecting sources of evidence (i.e., screening and eligibility) included in the scoping review.                                                                                                                                                     | 3-4 |

|                                                       |             |                                                                                                                                                                                                                                                                                                            |                           |
|-------------------------------------------------------|-------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|---------------------------|
| Data charting process‡                                | 10          | Describe the methods of charting data from the included sources of evidence (e.g., calibrated forms or forms that have been tested by the team before their use, and whether data charting was done independently or in duplicate) and any processes for obtaining and confirming data from investigators. | 3-4                       |
| Data items                                            | 11          | List and define all variables for which data were sought and any assumptions and simplifications made.                                                                                                                                                                                                     | 3-4                       |
| <b>SECTION</b>                                        | <b>ITEM</b> | <b>PRISMA-ScR CHECKLIST ITEM</b>                                                                                                                                                                                                                                                                           | <b>REPORTED ON PAGE #</b> |
| Critical appraisal of individual sources of evidence§ | 12          | If done, provide a rationale for conducting a critical appraisal of included sources of evidence; describe the methods used and how this information was used in any data synthesis (if appropriate).                                                                                                      | 3                         |
| Synthesis of results                                  | 13          | Describe the methods of handling and summarizing the data that were charted.                                                                                                                                                                                                                               | 3-4                       |
| <b>RESULTS</b>                                        |             |                                                                                                                                                                                                                                                                                                            |                           |
| Selection of sources of evidence                      | 14          | Give numbers of sources of evidence screened, assessed for eligibility, and included in the review, with reasons for exclusions at each stage, ideally using a flow diagram.                                                                                                                               | 4-8                       |
| Characteristics of sources of evidence                | 15          | For each source of evidence, present characteristics for which data were charted and provide the citations.                                                                                                                                                                                                | 4-8                       |
| Critical appraisal within sources of evidence         | 16          | If done, present data on critical appraisal of included sources of evidence (see item 12).                                                                                                                                                                                                                 | 8-9                       |
| Results of individual sources of evidence             | 17          | For each included source of evidence, present the relevant data that were charted that relate to the review questions and objectives.                                                                                                                                                                      | 4-8                       |
| Synthesis of results                                  | 18          | Summarize and/or present the charting results as they relate to the review questions and objectives.                                                                                                                                                                                                       | 4-8                       |

| DISCUSSION          |    |                                                                                                                                                                                                 |      |
|---------------------|----|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------|
| Summary of evidence | 19 | Summarize the main results (including an overview of concepts, themes, and types of evidence available), link to the review questions and objectives, and consider the relevance to key groups. | 9-14 |
| Limitations         | 20 | Discuss the limitations of the scoping review process.                                                                                                                                          | 15   |
| Conclusions         | 21 | Provide a general interpretation of the results with respect to the review questions and objectives, as well as potential implications and/or next steps.                                       | 15   |
| FUNDING             |    |                                                                                                                                                                                                 |      |
| Funding             | 22 | Describe sources of funding for the included sources of evidence, as well as sources of funding for the scoping review. Describe the role of the funders of the scoping review.                 | N/A  |

From: Tricco AC, Lillie E, Zarin W, O'Brien KK, Colquhoun H, Levac D, et al. PRISMA Extension for Scoping Reviews (PRISMA ScR): Checklist and Explanation. Ann Intern Med. 2018;169:467–473. doi: 10.7326/M18-0850.

Annexe 2

❑ **Search on Scopus database for SCHOOLS:**

( schools\* OR education\* OR academy OR college OR university AND students OR pupils ) AND ( mental AND health OR physical AND health OR wellbeing\* OR wellbeing AND pro motion OR wellness OR psychosocial OR psychological ) AND ( investment OR decision AND - making OR managing\* OR allocation\* OR resources\* OR time AND budget\* OR funds ) A ND ( evidence OR science OR research OR information OR experiences OR data OR results OR knowledge\* OR costs-benefits OR costs-effectiveness\* )

International Journal of Organization Theory & Behavior